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February 16, 2026

The Hon. Robert F. Kennedy Jr.
Secretary, U.S. Department of Health and Human Services
Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology
Mary E. Switzer Building
330 C Street SW
Washington, DC 20201.

Submitted electronically

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care; HHS Health Sector AI RFI [RIN 0955-AA13]

Dear Secretary Kennedy:

On behalf of Advocate Health, we appreciate the opportunity to comment on the recently published Request for Information: Accelerating the Adoption and Use of Artificial Intelligence (AI) as Part of Clinical Care.[1] Our full comments are outlined below.

About Advocate Health

Advocate Health is a leading nonprofit integrated health system in the United States providing care under the names Advocate Health Care in Illinois; Atrium Health in the Carolinas, Georgia, and Alabama; and Aurora Health Care in Wisconsin. Advocate Health is a national leader in clinical innovation, health outcomes, consumer experience, and value-based care, with Wake Forest University School of Medicine serving as the academic core of the enterprise. Headquartered in Charlotte, North Carolina, Advocate Health serves approximately six million patients and is engaged in hundreds of clinical trials and research studies.

Advocate Health is nationally recognized for its expertise in cardiology, neuroscience, oncology, pediatrics, and rehabilitation, as well as organ transplants, burn treatments, and

[1] 90 FR 60108

specialized musculoskeletal programs. Advocate Health employs an estimated 167,000 team members across 69 hospitals and more than 1,100 care locations and offers one of the nation's largest graduate medical education programs with more than 2,000 residents and fellows across more than 200 programs. Additionally, Advocate has five skilled nursing facilities with 572 licensed beds. Committed to ensuring access to care for all, Advocate Health provides more than $6 billion in annual community benefits.

Background

a. AI Regulation: How Current HHS Regulations Impact AI Adoption for Clinical Care

The primary federal regulatory frameworks affecting the adoption of AI in clinical care are Health Insurance Portability and Accountability Act (HIPAA) regulations and the Food and Drug Administration's (FDA) oversight of medical devices. To create an environment that supports responsible and scalable AI integration in healthcare, federal policymakers should modernize HIPAA to address contemporary data sharing practices, strengthen privacy and security protections, and provide clearer guidance on the appropriate use of patient data for AI development and deployment.

Federal legislation and regulation should set clear, consistent national standards for liability, accountability, transparency, and responsible AI use in both clinical care and operations. These standards should include a uniform definition of "artificial intelligence" and distinguish regulatory expectations across different AI types, including machine learning tools, computer vision systems, and large language models.

Because states regulate medical licensure, federal policymakers should also outline the appropriate boundaries for state level AI laws. States should retain the ability to regulate clinical practice, but within a federal framework that prevents conflicting or overly restrictive rules. This alignment is essential for multistate health systems and nationwide AI development.

b. Reimbursement: Policy Changes to Ensure Payers Incentivize Access to High Value AI Clinical Interventions

The cost of implementing AI solutions, including the necessary infrastructure for monitoring, governance, model evaluation, workforce training, and cybersecurity presents a significant barrier for small, rural and critical access hospitals as well as providers serving areas with significant socio-economic needs. Without targeted reimbursement policies or federal financial support, these organizations will be unable to adopt emerging AI technologies, even as such tools become standard components of high-quality care. As a result, disparities in access to advanced clinical tools may widen hampering one of this administration's key goals of improving population health outcomes.

Federal policy should ensure that payers have both the incentive and the operational ability to support access to clinically validated, high value AI tools. This may include establishing clear reimbursement pathways for AI-enabled services, offering supplemental payments or grant-based support for under-resourced providers, and creating mechanisms to evaluate the cost effectiveness and clinical benefit of AI tools across diverse care settings.

c. Research and Development: Opportunities for HHS Investment to Advance AI Integration in Care Delivery

HHS has a critical role in supporting research and development efforts that advance the safe, equitable, and evidence-based integration of AI into clinical care. Strategic investments should focus on:

- Supporting research into AI models that address social determinants of health and structural inequities.
- Funding pilot programs that test AI applications in under resourced settings, including rural and safety net providers.
- Advancing long-term R&D initiatives aimed at improving population health, care delivery efficiency, and equitable access to emerging technologies.

<u>Specific Questions</u>

In this section, we present our detailed responses to HHS's specific questions from the RFI.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Barriers to Clinical Use of Large Language Models

Several significant barriers currently limit the safe and effective use of large language models (LLMs) in clinical care. First, these systems may generate inaccurate or fabricated information ("hallucinations"), creating risks if clinicians rely on outputs without sufficient validation. LLMs may also omit critical clinical details or fail to surface relevant information, particularly when inputs are incomplete or ambiguous.

Second, there is no clear or consistent framework governing liability when an AI system provides misleading, incomplete, or unsafe recommendations. This uncertainty poses challenges for health systems, clinicians, and vendors, and may slow adoption of otherwise promising tools.

Third, the financial burden associated with implementing LLM-based solutions—including vendor licensing fees, internal development costs, technical infrastructure, and ongoing monitoring and governance—is substantial. These costs are especially prohibitive for small, rural, and resource constrained healthcare organizations, raising concerns about widening disparities in access to advanced clinical technologies.

At Advocate Health, we mitigate these issues by leveraging retrieval-augmented generation (RAG) and small language models (SLMs) and integrating our core protocols to limit hallucinations. Additionally, with these guardrails in place, the use of these LLMs results in a sensitivity that is better than human performance alone. For smaller organizations, these guardrails may not be easily implemented.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

HIPAA, Privacy, and Security Considerations

HIPAA should be reviewed and modernized to address privacy and security implications related to transmitting protected health information to technology companies such as OpenAI, Anthropic, and Google. As these entities increasingly handle sensitive patient data, including information from medical records, wearables, and patient generated sources, they

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should be required to adhere to the same privacy, security, and compliance standards applied to traditional healthcare providers and certified health IT systems.

Additionally, the use of highly regulated data—such as Medicare Shared Savings Program (MSSP) information—is governed by strict program requirements and contractual obligations. These restrictions may limit the ability of providers and Accountable Care Organizations (ACOs) to use such data in AI solutions or even to store it within AI-enabled platforms intended for internal use. Updated guidance is needed to clarify permissible uses of regulated data in AI development and operational workflows, particularly for organizations participating in federal value-based programs.

FDA Oversight and Regulatory Alignment

On January 6, 2026, FDA published revised final guidance documents on Clinical Decision Support Software ("Revised CDS Guidance") and its General Wellness Policy for Low Risk Devices ("Revised General Wellness Guidance"). FDA's revised CDS and General Wellness guidance documents directly address industry concerns about overregulation by broadening the categories of digital health products the agency chooses not to regulate, either based on its interpretation of statutory authority or as a matter of policy. We commend the FDA for publishing these updated guidance documents.

However, FDA's regulatory approach should be considered jointly with broader federal AI policies to ensure coherence, minimize duplication, and create predictable expectations for developers, clinicians, and health systems. Clear definitions, risk-based frameworks, and coordinated interagency guidance would significantly improve the regulatory environment for clinical AI.

AI Use by Payers and Impacts on Care Access

Legislation and regulation must also account for the increasing use of AI by payers to determine prior authorizations, coverage approvals, and claims denials. These tools exert direct and significant influence on the clinical care that patients receive. To protect patients and promote accountability, payers using AI for coverage determinations should be required to maintain transparency regarding:

- The role of AI in claims decisions,
- Rates of denials, appeals, and overturns,

- The specific AI models or algorithms used, and
- Demonstrated cost efficiencies achieved through AI.

Further, any financial savings attributable to AI-enabled administrative efficiencies should be expected to translate into lower premiums or reduced cost sharing for beneficiaries rather than solely accruing to payer institutions.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Liability Issues

Even when AI tools are not regulated as medical devices, they increasingly influence real clinical and social service decisions. This creates practical and legal challenges, particularly for safety-net providers and community-based organizations that lack extensive compliance or legal support. In community health settings, significant uncertainty remains regarding liability. Clinicians and community partners are often unsure whether they face increased exposure if they rely on an AI-generated recommendation that results in harm, or if they decline to follow such a recommendation and harm nevertheless occurs. These ambiguities place disproportionate burdens on federally qualified health centers, rural hospitals, and faith-based or nonprofit programs that already operate with limited legal resources and heightened perceived risk.

HHS regulations could provide meaningful clarity by:

- Affirming that reasonable reliance on validated clinical decision support tools does not, by itself, create new or expanded liability.
- Encouraging malpractice and institutional liability frameworks to distinguish clearly among clinician errors, organizational governance failures, and defects in the AI tools themselves.

Privacy Issues

Many AI developers, data brokers, and digital health platforms operate outside the scope of HIPAA yet routinely process highly sensitive health and social information. When failures or misuse occur, lines of responsibility are often unclear, leaving patients uncertain about where to seek recourse. To address these gaps, HHS could:

- Require clear documentation of roles and responsibilities among AI developers, healthcare organizations, and vendors.
- Extend HIPAA-aligned privacy and security standards to key non-covered entities that participate in healthcare-related AI activities.
- Establish transparency requirements so patients and caregivers can readily identify who is accountable for AI-driven recommendations and decisions at each stage of care.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Metrics and Evaluation Frameworks for AI

AI performance metrics should remain locally defined, as their relevance depends on how each tool is integrated into clinical workflows. Federal regulation of specific metrics would be overly prescriptive and could hinder innovation. However, federal guidance should encourage the use of governance structures, such as Advocate Health's Framework for the Appropriate Implementation and Review of AI (FAIR AI)[2] principles or internal AI oversight committees, to ensure organizations systematically evaluate AI tools for safety, equity, and alignment with patient-care priorities.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Limiting Additional Accreditation or Credentialing Requirements

[2] Wells BJ, Nguyen HM, McWilliams A, Pallini M, Bovi A, Kuzma A, Kramer J, Chou SH, Hetherington T, Corn P, Taylor YJ, Cuison A, Gagen M, Isreal M; FAIR-AI Consortium. A practical framework for appropriate implementation and review of artificial intelligence (FAIR-AI) in healthcare. NPJ Digit Med. 2025 Aug 11;8(1):514. doi: 10.1038/s41746-025-01900-y. PMID: 40790350; PMCID: PMC12340025.

Efforts to promote innovative and effective use of AI in healthcare should not rely on creating new accreditation, certification, or credentialing requirements. These processes often impose substantial administrative and financial burdens without delivering meaningful improvements in quality or safety. Registry abstraction and reporting already represent significant expenses for healthcare organizations, and federal and state agencies, as well as accrediting bodies, should be constrained from adding additional requirements that further increase administrative load without demonstrable value.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Areas of Highest Potential Impact for AI in Healthcare

AI has the greatest potential to drive meaningful improvements in healthcare by enhancing administrative efficiency, strengthening workforce capacity, accelerating research, and enabling more precise and timely personalized care. High-impact administrative applications include billing, supply chain optimization, and clinical data abstraction. Workforce supporting tools, such as those for scheduling, documentation, and radiology, can reduce burden and improve productivity. In addition, AI can significantly advance research and development efforts and support more individualized, data driven clinical decision making.

AI can assist with clinician and patient satisfaction. By supporting tools such as ambient listening and chart summarization, reducing documentation burden, improving efficiency, AI tools enable clinicians to work at the top of their license. These capabilities not only free clinicians to spend more meaningful time with patients but also help health systems manage ongoing workforce shortages more effectively

Shortcomings of current AI tools

- **Analysis of social drivers:** In our community health portfolio, we routinely encounter patients who fully understand their care instructions, adhere to prescribed medications, attend follow-up appointments, and make recommended lifestyle changes, yet remain unable to carry out their care plans due to significant social barriers. Many lack reliable transportation to medical visits. Others cannot afford their medications even with insurance coverage. Some do not have stable housing or consistent access to nutritious food. Many work multiple jobs with limited ability to take time off for medical needs.

  - Current clinical AI tools do not appear to incorporate these critical social factors—such as the availability of community resources, individual financial constraints, or the practical feasibility of care recommendations—into their assessments or outputs. These systems may identify a patient as high risk for hospital readmission, but they cannot determine whether that same patient has the means to attend a follow-up appointment or afford necessary prescriptions once insurance benefits are exhausted.

- **Linguistic/cultural gap:** Most existing AI systems are trained predominantly on English language data and interactions from majority populations. While translation functionalities may technically be available, these models often lack the cultural, linguistic, and contextual grounding necessary to generate recommendations that are meaningful or actionable in patients' day-to-day lives.

Novel AI Tools with the Greatest Potential

An integrated tool for social determinants of health (SDOH) screening and resource navigation represents one of the most impactful opportunities for next-generation clinical AI. Such a system could synthesize individual clinical data with community level indicators, such as food access, housing stability, transportation availability, and other relevant social determinants, to assess patient needs accurately and predict the likelihood of successful care plan completion. Importantly, this assessment would extend beyond clinical risk factors to incorporate material, social, and logistical barriers that frequently impede adherence.

A tool of this kind could automatically match patients with real time community resource availability and generate referrals to appropriate social services, community-based organizations, and financial assistance programs. Ideally, it would also track resource utilization and identify unmet needs or systemic gaps, thereby informing both individualized care planning and longer-term community investment strategies. In this model, AI would augment, rather than replace, the critical work performed by community health workers and care navigators by equipping them with data driven insights and automated administrative support.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Roles, Decision Makers, and Governance Structures

The appropriate roles and governance structures for AI adoption vary based on the size and complexity of a healthcare organization. Large systems like Advocate Health typically employ an AI or Digital Health Officer and maintain data governance and AI governance committees, national and specialty service lines, clinical informatics teams, and IT departments responsible for implementing models and vendor solutions.

AI deployment should be guided by the organization's strategic priorities, with consideration of both immediate and long-term value. Returns on AI investment should be assessed not only through short-term financial metrics but also through broader impacts on the patient and provider experience, access to care, and overall system performance.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Ensuring Data Access and Transparency in AI Development

Given the near monopoly among a few major electronic health record (EHR) vendors, these systems should permit greater interoperability and connectivity with AI developers, including health systems conducting internal AI development. Currently, EHR vendors control tightly how data can be exported and integrated into organizational data platforms. As a result, even large systems such as Advocate Health face constraints on how their own data can be accessed, processed, and used to develop advanced AI solutions, including those that rely on machine learning and large language models. This limits innovation and slows progress toward more effective, data driven care.

Federal policy could also establish a transparent framework governing the quality and risk profile of data used to train large language models (LLM). Such a framework would clearly indicate whether training data were scraped from the internet, derived from medical records, or sourced from clinical research—like how scientific evidence is graded. Greater transparency would allow healthcare organizations to better assess the reliability, safety, and appropriateness of LLM based tools used in clinical or operational settings.

Enhanced interoperability represents a critical lever for expanding market opportunities. Current federal interoperability requirements are primarily focused on read-only access to individual patient-level data, which constrains both the scope and scale of AI applications.

While these application programming interfaces (API) facilitate patient-mediated data exchange and point-in-time clinical insights, they do not reliably support population-level data aggregation needed for model training.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patient Privacy and Concerns About Agentic AI

Patients express significant concerns about the privacy of video, audio, and personal health data, as well as the risk that agentic AI systems may provide incorrect or biased information. Updating and modernizing HIPAA would help address these concerns by strengthening protections for emerging data types and clarifying expectations for the safe, transparent use of AI in clinical and patient facing settings.

Conclusion

In conclusion, we would like to highlight our key comments below.

- Given the rapid pace at which AI solutions are emerging in healthcare, alongside a continuously expanding field of technology vendors and a growing patchwork of state specific legislation, there is an urgent need for clear, coordinated federal action.
- Federal leadership is essential to modernize HIPAA, increase transparency for AI used by payers, and prevent interoperability barriers created by EHR vendors.
- Adoption must focus on clinical value and long-term strategic benefit, not additional administrative burden.
- AI's highest near-term impact lies in administrative simplification, workforce support, personalized care, and research acceleration.
- Healthcare organizations need flexible governance structures, not prescriptive federal mandates, to evaluate AI tools in alignment with organizational strategy and patient outcomes. Adopting a risk-based framework like Advocate Health's FAIR-AI enables responsible, scalable AI use while avoiding rigid rules that can stifle innovation and slow adoption.

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Thank you for the opportunity to provide feedback on HHS's Health Sector AI RFI. If you have any questions about our comments or need any additional information, please do not hesitate to contact Sabra Rosener, VP, Federal Affairs and Government Policy (Sabra.Rosener@aah.org). We stand ready to serve as a resource as you continue your important efforts to accelerate the adoption and use of AI as part of clinical care.

Sincerely,

James A. Crowder
Chief Technology and AI Officer